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June 16, 2025
Dr. Mehmet Oz
Administrator for Centers for Medicare & Medicaid Services (CMS)
U.S. Department of Health and Human Services
7500 Security Boulevard
Baltimore, Maryland 21244-1850

Re: Wellth Comments on the CMS Health Technology Ecosystem Request for Information (CMS-0042-NC)

Submitted via regulations.gov

Dear Administrator Oz:

On behalf of Wellth, we are pleased to submit comments in response to the Centers for Medicare & Medicaid Services (CMS) Health Technology Ecosystem Request for Information (RFI). As a leading digital health company, Wellth partners with plans and payers to improve health outcomes and quality of care for high-cost, high-risk patients through innovative care management programs.

We appreciate the opportunity to submit detailed comments and support CMS' leadership in seeking to modernize the digital health ecosystem and improve Medicare beneficiaries' access to innovative health technologies.

Our detailed comments include specific recommendations for CMS on how best to increase access to proven digital health management applications, strengthen interoperability, reduce barriers to the seamless exchange of health information, and accelerate the process toward value-based care.

As requested, Wellth's detailed comments are organized in response to the specific questions included in the RFI. Please also find our video submission linked **here**.

We appreciate the opportunity to share our perspectives and insights and look forward to partnering with CMS to advance the shared goal of improved Medicare beneficiary access to innovative and effective digital health management tools.

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Wellth detailed comments:

Patients/Caregivers domain:

PC-3. *Are you aware of health management, care navigation, or personal health record apps that would be useful to Medicare beneficiaries and their caregivers?*

Yes. Wellth is a proven digital health management solution tailored for Medicare and Medicaid beneficiaries with complex care needs—those living with multiple chronic conditions, low historical adherence, and high utilization of costly services like inpatient stays and emergency department visits.

By combining behavioral science, strategically aligned incentives, real-time daily engagement, and integration with care plans, Wellth empowers individuals to take control of their health. This approach increases meaningful engagement with providers and caregivers, driving better outcomes and reducing avoidable costs.

To build a **people-centered**, **technology-driven** healthcare system, **individual beneficiaries** must also have access to interoperability. While CMS continues to build the critical infrastructure for interoperability - such as provider directories, verification processes, Blue Button 2.0 expansion, Point of Care enhancements, and improved data exchange – additional efforts are necessary to promote Medicare beneficiaries' access to clear, personalized pathways to engage with their health and these tools.

This is where Wellth plays a vital role: bridging the gap between the infrastructure CMS is facilitating and the individuals most in need of engagement. By meeting people where they are, with thoughtful behavioral design, aligned incentives, and trusted daily

connections - Wellth ensures that even the most disengaged populations are supported, connected, and empowered to improve their health.

Working in partnership with Managed Care Organizations—including national payers, integrated delivery networks, and local health plans, Wellth uses claims and encounter data to identify and engage individuals most likely to benefit from personalized digital support. Once enrolled, users receive a tailored experience in the app that supports a range of daily self-management behaviors, including medication adherence, blood pressure and glucose monitoring, and healthy meal tracking.

These behaviors are reinforced through immediate, small-dollar incentives—applying proven principles of behavioral economics to drive habit formation and long-term engagement. As users demonstrate consistency, their experience expands to include broader preventive and condition-management goals such as health risk assessments and preventative screenings.

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Today, Wellth supports over **50,000 active daily users**, 90% of whom consistently complete their assigned self-management tasks. Over the past decade, our users have completed

more than **40 million daily check-ins**—real actions that build healthier habits and improve quality of life. Many of our most engaged users are in their 80s and 90s, with some even over 100 years old—demonstrating that digital tools, when thoughtfully designed, can successfully engage people across age groups and comfort levels with technology.

This consistent engagement translates into meaningful results. Wellth users demonstrate an average of **90% daily adherence to care plans**—even among those previously considered non-adherent. This unprecedented level of engagement in healthcare leads to significant clinical and financial outcomes:

- A **43% average reduction in high-cost utilization**, including a **51% decrease in inpatient admissions** and fewer avoidable emergency department visits.
- **Increased use of primary care, specialty care, and pharmacy services**—a "right-sizing" of utilization that shifts care to the right place at the right time.
- Measurable quality gains, including **consistent 4-Star or higher ratings among Medicare plan partners across Part C and D measures**, driven in part by a **16% average increase in Proportion of Days Covered (PDC)**, a key indicator of medication adherence.

Ultimately, our payer partners report a 2–4x return on investment within the first year, driven by cost-of-care reductions, improved quality scores, and more engaged, healthier members.

By lowering barriers to adherence and empowering individuals with daily, personalized support, Wellth advances the goals of value-based care—delivering better outcomes, greater efficiency, and improved quality of life for those who need it most.

Expanded Use Case Potential for the Wellth App

Wellth's current business model centers on partnering with payers to improve outcomes for high-risk, high-cost members—primarily through chronic disease management, medication adherence and preventive actions. However, the Wellth app has the potential to serve a much broader segment of the Medicare population, particularly individuals who struggle to consistently manage their health and navigate the healthcare system.

With a modest shift in implementation approach, Wellth's personalized, behaviorally-driven app could support a wider range of CMS priorities. For example, if CMS aimed to drive specific member actions—such as completing wellness visits, closing preventive care gaps, or using supplemental benefits—Wellth's adaptive app experience could be configured and effectively leveraged to meet those goals.

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The app fosters a daily, high-touch relationship with users, creating trust and sustained engagement that CMS could leverage to deepen its connection with beneficiaries and plug them into the infrastructure CMS is building.

As interoperability advances and more real-time data becomes available, Wellth could be used to deliver highly personalized prompts and support across multiple domains, including:

- **Preventive Care:** Prompting beneficiaries to schedule and complete annual wellness visits, screenings (e.g., mammograms, colonoscopies), or immunizations.
- **Care Navigation:** Helping individuals understand and use supplemental benefits (e.g., transportation, food, OTC) they may not be aware of or know how to access.
- **Chronic Condition Support:** Reinforcing self-management routines for conditions like diabetes, hypertension, or COPD using tailored daily nudges, education, and barrier resolution (e.g., medication pickup reminders, motivational support).
- **Health Data Engagement:** Encouraging beneficiaries to access, understand, and use their own health records—especially as APIs and FHIR standards increase access across providers and plans.

Ultimately, Wellth's technology could support CMS in advancing person-centered care by increasing beneficiary engagement, and improving quality measures across Medicare programs—not just for the highest-risk groups, but at scale.

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PC-4. *What features are missing from apps you use or that you are aware of today?*

A. *What apps should exist but do not yet? Why do you believe they do not exist yet?*
B. *What set of workflows do you believe CMS is uniquely positioned to offer?*

Most engagement efforts and healthcare apps today lack the ability to drive consistent, long-term behavior change—especially among individuals with chronic conditions and complex needs. While many solutions aim to support health management, they often fall short in bridging the gap between intent and action on a daily basis and sustained over time. In addition, many apps aiming to support chronic disease management tend to focus on singular tasks or conditions, rather than addressing the root cause—individual behaviors.

Unlike consumer platforms—which successfully create daily habits through gamification, personalization, and intrinsic motivation—many health apps rely on transactional engagement: reminders to schedule appointments, refill medications, or complete forms. These touchpoints are often payer-driven and fail to resonate with individuals on a personal level to drive meaningful engagement. And most importantly, they do not create intrinsic motivation for beneficiaries to form better habits that improve their health management. Additionally, if the individual is required to pay for the solution, uptake is limited, especially for beneficiaries who are most at-risk.

Critically, most apps:

- Lack daily habit formation features that are proven to drive sustained care plan adherence and self management.
- Do not personalize content or experiences based on the user's actual conditions, behavior or motivation.
- Use inconsistent definitions of "engagement" and "retention," which makes it difficult to compare impact across solutions.
- Are not well-suited to individuals facing social, behavioral, or cognitive barriers—those who arguably need support the most.

A. What apps should exist but do not yet? Why do you believe they do not exist yet?

While many digital health apps already exist, most suffer from low uptake and fail to demonstrate sustained engagement. Moreover, these solutions are typically used by beneficiaries who are already highly engaged in managing their health.

Wellth's experience helping tens of thousands of Medicare Beneficiaries reshape their self-management habits using proven behavioral applications are innovative and highly effective. This RFI presents an exciting opportunity for us to share that experience with CMS. So, perhaps we can reframe the question to: **What is already working and why aren't more**

beneficiaries managing their health effectively on their own using proven solutions? The answer may not be more apps or tools.

Wellth has already proven its ability to:

- Drive daily healthy actions related to chronic disease management and prevention through a personalized app experience
- Lower the costs associated with chronic disease by leveraging exceptional engagement, driven by behavioral science and behavioral economics
- Drastically improve self-management and preventive behaviors—even among the most complex, high-risk beneficiaries
- Create a high-touch relationship with individual beneficiaries, enabling real-time, bi-directional feedback that supports prevention and other health interventions

By motivating individuals and changing behaviors, the Wellth app has demonstrated that effective self-management is possible through a digital solution. With broader access, Wellth could scale effective chronic disease management and prevention efforts nationwide. By increasing adoption and improving interoperability of this proven solution, CMS has a powerful opportunity to significantly reduce the crippling costs of chronic disease while empowering individuals to take control of their health. Examples of ways to increase access and interoperability are shared in PC5.

B. What set of workflows do you believe CMS is uniquely positioned to offer?

CMS is uniquely positioned to:
Standardize outcome measures related to digital engagement and define consistent terms for "engagement," "retention," and "behavior change."

Incentivize proven digital solutions like Wellth that directly benefit individuals—especially those that demonstrate daily, high-touch engagement leading to improved outcomes.

- Remove barriers for beneficiaries by creating reimbursement pathways for solutions that focus on patient-centered behavior change, not just clinical care coordination.
- Champion models that can incorporate caregivers and support whole-person care, especially in complex populations.

In short, CMS has the power to reframe digital health from a payer-centered model to one that prioritizes patient-centered empowerment, behavior-driven value-based care—supporting solutions like Wellth that have proven daily engagement, high retention, and measurable outcomes for complex Medicare beneficiaries.

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PC-5. *What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?*

A. *What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality or impact or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process? What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)?*
B. *What changes would enable timely access to high quality CMS and provider generated data on patients?*

To drive meaningful adoption and sustained use of digital health products among Medicare beneficiaries and their caregivers, CMS has the opportunity to remove barriers to accessing proven digital solutions by focusing on incentive alignment, trust-building, and relevance to individuals' daily lives. While access to technology is important, proven engagement is the key differentiator—especially for individuals managing chronic conditions, facing social barriers, or historically disengaged from care.

What works: Wellth's experience demonstrates that when beneficiaries are directly incentivized to complete small daily health actions—such as taking medications, checking blood pressure, or completing preventive screenings—adoption is significantly higher and long-term engagement is sustained. Over 80% of Wellth's Medicare users remain active daily and adherent one year after enrollment, a benchmark rarely achieved in digital health. To encourage patient and caregiver engagement at scale, CMS could:

- Incentivize beneficiary participation in digital health programs that demonstrate behavior change and measurable outcomes—not just provider or health plan compliance.
- Encourage co-design of improvements to existing solutions with the populations they serve, ensuring cultural and usability relevance and expanded use cases.
- Promote caregiver-inclusive models that recognize and support the critical role caregivers play in day-to-day care management.
- Fund digital literacy and access programs that reduce technical barriers for older adults and underserved communities.

Rather than a formal approval process—which may inadvertently slow innovation—CMS could establish a **"CMS Validated"** or **"CMS Endorsed"** designation for digital solutions that meet clearly defined criteria aligned with Medicare's goals. This endorsement could guide payers, providers, and beneficiaries toward products that have demonstrated efficacy and interoperability, without adding regulatory burden or having to create unnecessary additional solutions.

Suggested CMS validation criteria might include:

- Proven impact on health outcomes, adherence, and/or cost reduction
- Demonstrated patient and caregiver engagement and satisfaction
- Support for prevention and chronic disease management
- Access considerations, including multilingual content, caregiver support, and non-smartphone accessibility
- Interoperability with EHRs and claims systems
- Compliance with privacy, HIPAA, and data governance standards

CMS can increase adoption through:

- Reimbursement adjustments (e.g., direct support for patient-facing tools via existing or new CPT codes). Payers usually limit access to proven tools like Wellth to 12 months. Many individuals would benefit from continued self-management with validated digital solutions. In fact, some payers have performed analyses of the cost impacts to removing their members from the Wellth program after 12 months. The result is that costs increase and members fall back into old habits.
- Inclusion of validated tools in chronic care management (CCM) or transitional care management (TCM) workflows. CMS could consider re-writing the justification for these codes to allow providers to "prescribe" a proven self-management solution like Wellth to patients who struggle with managing their conditions or overall health.
- To track efficacy, CMS might add a flag on provider claims, enabling physicians to indicate when a patient is using a digital health management solution. Embedding this process into physician workflows and integrating it with EHR systems could remove barriers, improve access, and reduce friction in getting effective self-management tools into the hands of beneficiaries who need them.
- Targeted communications to beneficiaries and caregivers, reinforcing how digital tools can support their personal health goals.
- Data sharing enhancements that allow digital tools to deliver more timely, personalized nudges and feedback to users.

To unlock the full potential of digital health engagement, solutions must be able to access timely and accurate data to personalize outreach, identify care gaps, and trigger proactive interventions. CMS can support this by:

- Accelerating real-time access to claims, encounter, and care coordination data for trusted third-party partners serving beneficiaries
- Standardizing APIs and FHIR adoption to facilitate secure, patient-consented data flow across systems
- Supporting data-sharing frameworks that allow care navigators and digital tools to receive timely updates on hospitalizations, medication changes, or new diagnoses
- Exploring caregiver consent models to allow authorized caregivers to access and act on behalf of patients within digital tools

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To improve health outcomes for Medicare beneficiaries, CMS has a unique opportunity to support the adoption of digital health solutions that prioritize sustained engagement of patients and caregivers. By setting clear standards for quality, usability, and integration—without limiting innovation—and by improving access to timely data and appropriate reimbursement models, CMS can help scale effective, evidence-based solutions like Wellth. These tools show that when individuals are motivated and supported in building daily healthy habits, the result is better health, stronger patient-provider relationships, and more efficient use of healthcare resources.

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PC-6. *What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?*

To make digital health tools accessible for Medicare beneficiaries—especially those with limited tech experience—simplicity, routine, and motivation are key.

What sets Wellth apart is our ability to consistently engage complex, hard-to-reach members through the science of behavioral economics. We meet members where they are—often starting with no prior app use—and build trust through motivating simple, daily routines paired with meaningful rewards.

Our app features that we have found make Wellth easy to use:

- A frictionless onboarding process with step-by-step guidance
- Minimal, intuitive design with large fonts, clear prompts, and limited navigation to reduce cognitive load
- Daily, bite-sized tasks focused on high-impact behaviors like medication adherence or vitals tracking
- Immediate incentives that reinforce habits and drive sustained behavior change
- Accessible caregiver support for those who need help navigating digital tools

The result is over 90% daily engagement among Medicare and Medicaid members—proving that when we design with human behavior in mind, digital tools can truly become accessible, effective, and life-changing. Wellth's experience supporting tens of thousands of Medicare beneficiaries in improving their self-management habits through proven behavioral science is truly distinctive. This RFI presents an exciting opportunity to potentially create a **single access point** for Medicare beneficiaries to manage their health. By integrating Wellth within the ecosystem- patients, caregivers, providers, payers and CMS could create a more patient-centered experience that incentivizes better health and outcomes.

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PC-7. *If CMS were to collect real-world data on digital health products' impact on health outcomes and related costs once they are released into the market, what would be the best means of doing so?*

To effectively collect real-world data on digital health products, CMS could establish a standardized, yet flexible, post-market surveillance framework that integrates with existing healthcare data infrastructure. Key components would include:

- **Partnerships with Health Plans and Providers:** CMS could require or incentivize health plans and providers participating in federal programs (e.g., Medicare Advantage, Medicaid MCOs) to report on the use and outcomes of digital health tools. This would enable collection of data across diverse populations, care settings, and geographic regions.
- **Integration with Claims and EHR Data:** Leveraging CMS's access to Medicare and Medicaid claims data and aligning it with EHRs and digital product data feeds (e.g., app engagement, task completion, biometric readings) would allow for a more complete picture of a product's impact on clinical outcomes, utilization, and costs.
- **Digital Health Product Registries:** CMS could fund or endorse condition-specific registries for digital health products, where developers and payers submit structured, de-identified data on utilization, clinical metrics (e.g., blood pressure, A1c), engagement, and retention. This would facilitate comparative analyses and ongoing evaluation.
- **Patient-Reported Outcomes and Surveys:** Incorporating regular patient-reported outcomes (PROs) and experience surveys into data collection can provide critical qualitative context—especially important for measuring behavioral, mental health, or social outcomes often influenced by digital interventions.
- **API and Interoperability Standards:** Mandating adherence to common interoperability standards (e.g., FHIR APIs) for digital health products will make real-world data easier to aggregate and analyze across different care systems and vendor platforms.
- **Pilot Programs and Public-Private Collaborations:** CMS could launch demonstration projects with digital health innovators to evaluate longitudinal outcomes under real-world conditions. These pilots would help refine data collection protocols and validate outcomes-based reimbursement models.

By taking a multi-pronged approach that combines quantitative claims and EHR data with qualitative patient-reported outcomes, CMS can build a robust evidence base for evaluating digital health innovations and supporting broader adoption of tools that demonstrably improve care and reduce costs.

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PC-8. *In your experience, what health data is readily available and valuable to patients or their caregivers or both?*

A. What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?
B. What are specific sources, other than claims and clinical data, that would be of highest value, and why?
C. What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?

Wellth's business model as it exists today is to partner with Payers (MCO's, at-risk provider groups etc.) to reach high-risk individuals within their membership to improve their health outcomes, one individual at a time.

The method for reaching these beneficiaries is through a lengthy sales process with the payer that usually hinges upon cost and quality improvements Wellth can achieve through better engagement and habit transformation with these risky individuals. Upon contracting with the client (Payer), Wellth gains access to some or all of the following data sets, with the goal of identifying the most complex, hard to reach individuals which Wellth's solution will provide the largest impact to cost and quality for the payer. Wellth then performs all the outreach necessary to inform beneficiaries of the program, assists them in downloading and using the app, and keeps the support live throughout the care journey. This creates high levels of engagement that is sustained over time. We've found that the following data sets have provided the best chance at identifying the most impactful individuals to target for the Wellth program:

- Medical Claims
- Pharmacy Claims
- Member Base data (eligibility)
- Encounter Data
- PBM Data to further identify low adherence
- In some instances, Provider integrations can be achieved via EHR integration

This business model, while challenging, costly and time consuming, provides the ability for beneficiaries to have the Wellth solution available to them at no cost, while also benefiting from the monetary reward structure of the app which creates intrinsic motivation over time. Often, clients (Payers) are either unwilling or unable to provide all data sources. Better data sharing capabilities and willingness would increase both the amount of individuals who could benefit from this proven solution and the effective monitoring of the impacts of the solution on total cost of care and quality improvements.

Other data feeds that would allow for more timely intervention and a more tailored and timely experience would be to have ADT (Admission, Discharge, Transfer) feeds integrated

with our clients. However, national coordination of this data is sparse. But more real-time data to feed into the Wellth app would allow for better experience and more ability to follow up with patients after discharge. Also, more timely information on member activity,

especially those that close care gaps with labs or provider exams. This typically takes Wellth weeks to months to discover. Better integration, which CMS is clearly pushing for, would be helpful for more timely and accurate user experiences.

An ideal situation could be that "**CMS approved or validated"** solutions would have easier pathways to the aforementioned data in a standard process and format. In our experience at Wellth, Payers can be very reluctant to share these data sets upon partnership, and most often insist on their own validation of the outcomes which is usually only financial considerations in short windows of time. If CMS could work with proven solutions like Wellth on validation of outcomes, this would help to identify true value to the ecosystem as well as the individuals improving their health.

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Providers Domain:

PR-1. *What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients?*

A. *What are the current obstacles?*
B. *What information should providers share with patients when using digital products in the provision of their care?*
C. *What responsibilities do providers have when recommending use of a digital product by a patient?*

In an ideal ecosystem, providers—including those in rural or under-resourced areas—would have a simple, supported way to refer patients managing chronic conditions to proven digital tools like Wellth. These tools can extend the provider's reach and reinforce daily self-management behaviors through accessible, incentivized support.

Current Obstacles:

- **Unclear payment pathways:** Providers often lack clarity on who pays for the tool—whether CMS, a payer, or the patient—and whether the tool is covered under existing benefits. Since the Payer is the usual decision maker for allowing access to no-cost digital tools for patients, access is limited. A solution to this problem could be that CMS modify its definition for what justifies billing for a chronic care management (CCM) code under the physician fee schedule to include the use of a proven digital health product like Wellth.
- **No central listing or vetting process:** Without a CMS-endorsed or validated list of approved digital tools aligned with specific conditions or goals, providers hesitate to refer patients due to uncertainty around quality and fit. Again, the same challenge of the payer-provider-patient relationship makes access a challenge.
- **Eligibility complexity:** Providers need clear, simple criteria to know which patients qualify for proven solutions, ideally integrated into existing workflows (e.g., EHRs).

What Providers Should Share with Patients:

- **Cost transparency:** Will the patient face any out-of-pocket cost?
- **Purpose and expected benefits:** What does the solution do, and how can it help improve the patient's condition and life?
- **User expectations:** What tasks will the patient be expected to complete (e.g., daily check-ins)?
- **Support access:** Who can the patient contact for technical or clinical help?
- **Cultural relevance:** Is the solution in-language and capable of supporting diverse populations?

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Provider Responsibilities:

- **Right-fit referrals:** Providers should understand the solution's general benefits and ensure alignment with a patient's specific needs and conditions.
- **Expectation-setting:** Providers should help patients understand the role of the digital tool in their care journey—including expected outcomes and what participation involves.
- **Ongoing support:** Providers don't need to manage the digital product day-to-day but should remain aware of its role in the broader care plan.

Recommendations for CMS:

CMS could significantly accelerate provider adoption by:

- Creating a vetted directory of a small number of validated digital tools aligned to specific use cases and goals. The directory should be limited, as too much "choice" can lead to confusion.
- Defining standard eligibility and referral workflows, potentially integrated into EHRs.
- Establishing flexible reimbursement models that support provider referrals without financial burden on patients or administrative burden on providers.

These steps would make it easier—and safer—for providers to engage patients in digital care pathways that extend beyond the clinic walls.

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Value Based Care Domain:

VB-1. *What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?*

For APMs and MSSP participants to adopt digital health management and care navigation tools more effectively, two foundational questions must be addressed:

1. Which tools are truly effective?
2. How are those tools sustainably paid for?

Today, most incentive models focus on the payer–provider relationship, while the patient—the individual who must engage daily for outcomes to improve—is often left out. Digital tools like Wellth demonstrate that when individuals are directly supported and incentivized, sustained behavior change is achievable, even among hard-to-reach Medicare populations.

However, the current digital health market is noisy and fragmented. Without clear standards, providers and APM participants are hesitant to adopt solutions due to uncertain ROI, operational complexity, and limited data-sharing infrastructure.

Key Recommendations for CMS:

- **Create a CMS-Endorsed Directory:** Establish a certification or endorsement process for digital tools that meet defined standards (e.g., measurable impact on outcomes, integration capability, patient-centered design, behavioral engagement).
- **Incentivize use in APMs:** Offer incentives for APM participants that deploy CMS-endorsed tools tied to specific use cases (e.g., chronic disease management, Prevention). These could be in the form of shared savings bonuses, upfront investment payments, or reduced reporting burdens.
- **Standardize Eligibility and Data-Sharing Requirements:** Define common eligibility criteria for digital tools and require payers in APM arrangements to share core datasets (claims, encounter, pharmacy) with solution providers under standard agreements.
- **Center the Individual:** Ensure that digital tool reimbursement models include incentives directly tied to beneficiary engagement and outcomes—not just provider behavior.

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Conclusion:

To unlock the full value of proven digital health tools for beneficiaries in APMs, CMS can play a catalytic role by clarifying what "works," enabling data access, and aligning incentives around individual behavior change—ensuring the right solution reaches the right person, at the right time. With the right approach, CMS can improve access to proven digital tools that can both prevent chronic illness and the progression of chronic diseases.

Sincerely,

Matthew Loper

Matt Loper
Chief Executive Officer, Co-Founder
Wellth